Running Gait Assessment – Intake & Screening Form

Complete sections 1–5 before the runner goes on the treadmill. Any ticked red flag in section 3 means stop and refer before a running assessment or loading programme.

1. Runner details

FIELD	DETAILS
Name	
Age	
Contact (phone / email)	
Date of assessment	
Clinician	
Referred by	
Height / weight	
Occupation / daily activity	

2. Presenting complaint

QUESTION	ANSWER
Where is the pain? (side, location; mark on body chart)	
When did it start?	
How did it start? (gradual / sudden; linked to a change in training?)	
Pain now (0–10)	
Worst pain in the past week (0–10)	
During a run: when does it start, and does it ease, stay or build?	
After a run: how long until it settles?	
Next morning: better, same or worse?	
Aggravating activities (running, hills, stairs, sitting, walking)	
Easing factors	
Treatment so far and its effect	

3. Red flag screen

Tick any that apply. **Any tick = no running assessment today; refer for further work-up (likely imaging).**

Bone stress injury

- ☐ Focal, pinpoint bony tenderness (rather than diffuse tenderness along a border)
- ☐ Pain at rest or at night
- ☐ Pain that now interferes with walking
- ☐ Pain that builds through a run and persists afterwards, rather than easing as the runner warms up
- ☐ Pain that doesn't settle with brief rest
- ☐ Painful single-leg hop on the affected side

General health

- ☐ Unexplained weight loss, fever or feeling generally unwell
- ☐ Night sweats, or pain unrelated to activity or position
- ☐ Pins and needles, numbness or weakness in the leg
- ☐ Recent trauma, a fall or a sudden onset with swelling
- ☐ History of previous stress fractures, low bone density, or missed or irregular periods

Action taken if any box is ticked:

4. Running and training history

QUESTION	ANSWER
Years running	
Current weekly volume (km or time)	
Weekly volume 4–6 weeks ago (any sudden increase?)	
Runs per week, and longest run	
Type of running (easy, intervals, hills, trail, track, races)	
Recent change in surface or terrain	
Footwear (model, age or km, recent change)	
Cadence if known (watch or app, steps per minute)	
Goal event or target, and date	
Cross-training and strength work	
Has anyone changed your running technique? (e.g. switched foot strike)	

5. Injury history and other factors

QUESTION	ANSWER
Previous running-related injuries (what, when, how they resolved)	
Previous imaging or surgery	
Other medical conditions and medication	
Sleep (hours a night, quality)	

QUESTION	ANSWER
Energy intake and recent weight change	
Work, study and family demands on training time	
Pressure from a race date, club, coach or running group	
Confidence in running now (0–10)	
Worried about re-injury? (yes / no, notes)	
What does the runner want from today?	

6. Physical screen

TEST	LEFT	RIGHT	NOTES
Single-leg squat x 10 (knee control, pelvic drop, trunk lean)			
Single-leg hop × 10 (pain 0–10)			
Single-leg calf raise (reps to fatigue, pain 0–10)			
Knee-to-wall dorsiflexion (cm from wall)			
Standing tibial varus (degrees)			
Hip abduction strength (side-lying, or dynamometer)			
Hip external rotation strength			
Palpation findings			
Special tests (condition-specific)			

7. Gait analysis record

SET-UP	DETAILS
Treadmill or overground	
Speed (km/h) and incline	
Footwear used	
Settling time before filming (min)	
Camera height, distance and frame rate	
Markers placed (ASIS, PSIS, greater trochanter, mid-lat-thigh, lat knee, mid-lat-tibia, lat malleolus)	

Sagittal view (side)

VARIABLE	LEFT	RIGHT	NOTES
Foot strike (rearfoot / midfoot / forefoot)			
Overstriding (foot ahead of hip at contact)			
Knee flexion at contact			
Trunk lean			
Vertical oscillation			

Posterior view (behind)

VARIABLE	LEFT	RIGHT	NOTES
Pelvic drop (corroborate: lower reliability by eye)			
Hip adduction / crossover			
Knee position (valgus / knee window)			
Heel whip			
Rearfoot eversion (corroborate: lower reliability by eye)			

Cadence

CADENCE	RESULT
Method (30 s count × 4, or watch)	
Steps per minute at test speed	

8. Clinical impression and plan

ITEM	NOTES
Working diagnosis (from history, palpation and special tests, not gait alone)	
Gait findings linked to this injury	
Main contributing factors (load, strength, mobility, footwear, other)	
Strength and loading programme	
Gait retraining needed? (yes / no; target, e.g. cadence +5–10%)	
Load modification (running volume, surfaces, rest)	
Return-to-run stage and next progression criterion	
Review date	

9. Body chart and additional notes

Mark the pain location (section 2) and any palpation findings.

FRONT

BACK

Additional notes

Clinician signature

Date